

Paw Patrol Team Brief

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We are building Paw Patrol to help an officer, a command desk and a receiving hospital share incident context. This brief explains the product idea, the updated dashboard workflow and how our teammates can run and extend it. The dashboard is a demonstration with fictional people and scripted signals; it does not contact police, ambulance services or hospitals.

The product idea

Our intended system combines body-worn camera and microphone signals, GPS and watch heart-rate data. Models could flag possible danger and the software could discreetly notify command and nearby officers, without relying solely on a spoken radio call. A separate injury report would start medical coordination and prepare a structured hospital handoff. We have not validated detection accuracy, clinical outcomes or response-time improvements.

What changed after the consult

The command view needs actionable context: what a reported person is wearing, visible items, where they were last observed and who reported it. A panic action requests assistance without claiming an injury. EMS can be assigned while a scene is unsafe, but must remain staged until an explicit command clearance report authorizes patient access in the demonstration.

The hospital view uses MIST: Mechanism of injury, Injuries reported, Signs and symptoms, and Treatments performed. Heart rate, blood pressure, pulse assessment and level of consciousness are separate observations. Unknown readings stay unknown.

Scope of the dashboard

Area	Demo capability	Not established by the demo
Workspaces	Command, Officer and Hospital views share a selected person and scenario.	Separate authenticated users or production access controls.
Map and body	Boston and Cambridge map with 3D buildings; interactive supplied human model.	Live navigation, a personal body scan or a clinical injury model.
Coordination	Panic, scene reports, staged EMS and local MIST records.	Real dispatch, hospital messaging or ATAK connectivity.
Signals	Synthetic heart rate and labeled scripted events.	Connected phone/watch feeds or validated model conclusions.

The **dashboard branch** is `codex/paw-patrol-dashboard`. Its standalone app belongs in `apps/paw-patrol` alongside the team's root application. Keeping these applications separate preserves the newer tracking and backend work on main while we agree on an integration contract.

What each workspace needs to show

All three workspaces are designed for a computer screen. Changing workspaces should retain the selected person and incident state. Changing people must update the identity, body-view session and applicable records without copying another officer's injury or medical notes.

Command and tactical context

Command combines the patrol roster, map, incident timeline and response desk. The tactical report describes a reported person separately from the selected officer. Clothing and visible items are observations, not evidence of identity or proof of a threat. A possible weapon signal remains unverified unless a person explicitly reports otherwise.

Report field	What the team should record
Clothing and appearance	Reported garments and distinguishing visible details; do not infer identity or protected characteristics.
Visible items	What was observed, including uncertainty. An unverified object is not a confirmed weapon.
Last observation	Reported location and the time of the observation, not an assumed current location.
Source and status	Scripted sample or demo operator, with verification and freshness stated.

Officer workspace and panic

The officer view shows the selected officer, surrounding map, assignment, sample heart-rate trend and generic body view. The panic control creates a manual assistance request for that officer. It does not infer a gunshot, an injury, loss of consciousness or a treatment need. Repeated activation should not create duplicate requests for the same person in one run.

Command can acknowledge the request. Acknowledgement means the alert was seen; it does not mean the scene is safe. The panic and acknowledgement should remain visible in the local event history. The current demo prevents new scene changes and panic actions after transport begins so it does not rewrite an earlier clearance or move a patient backward.

Hospital and EMS workspace

The hospital case opens only after the explicit scripted injury report for P-01. It combines MIST observations, the reported scene state and transport context. A different selected officer stays on standby unless that person has a qualifying report. The body is a generic visual reference; the script does not identify a wound location.

A copied MIST summary is a local demo artifact. Copying it does not notify a hospital, request treatment or send patient data to an external service.

The demonstration and scene safety

The default sequence runs for 90 seconds. These are elapsed demonstration timestamps, not measured response performance or clinical deadlines. Every dispatch, movement and handoff below is simulated.

Time	Event	Visible consequence
00:00	Patrol ready	Four fictional officers; no active injury case.
00:15	Possible weapon signal	Unverified camera sample for P-01; scene marked unsafe.
00:22	Audio concern	A scripted assistance signal joins the timeline.
00:30	Backup assigned	Sample units move toward the incident.
00:45	Injury reported	A scripted gunshot report creates P-01's medical case. Site and clinical status remain unknown.
00:52	EMS assigned	EMS-01 stages outside the incident. Assignment does not permit entry.
00:56	Command clearance report	The default script records a distinct clearance report, authorizing patient access in the demo.
01:00	Transport begins	The sample transport starts only with clearance recorded.
01:15	Receiving stage	The local hospital view displays the available handoff.
01:30	Handoff complete	Playback stops. Reset begins a fresh demonstration.

How to demonstrate the safety gate

Start the sequence in Command. Before transport, record an unsafe or unknown scene status. That manual report overrides the default scripted clearance. When playback or Next stage tries to cross into transport, the demo stops at 00:59 and EMS remains staged. Record clearance explicitly, then resume or advance to continue.

A panic request also reports an unsafe scene and pauses playback. Acknowledging the panic does not remove the scene gate. Reset clears the run's manual reports, panic state and clinical entries. The normal automatic storyline still includes its own separately labeled command clearance event.

What the gate does not claim

The software does not assess whether a real scene is safe and does not instruct officers to use force. Real deployment would require authorized command, local operating procedures, reliable acknowledgements and an auditable external dispatch process.

The MIST medical handoff

MIST structures the information passed to a receiving team. It is a communication format, not a diagnosis or a treatment recommendation. In this product, I means Injuries; interventions belong under T with treatments. Only observed, reported or explicitly simulated data should populate the record.

Part	Content	Default in this demonstration
M Mechanism	How the injury was reported to occur.	Reported gunshot from the explicit scripted event.
I Injuries	Reported injuries and anatomical site, if known.	No injury site, severity or diagnosis supplied.
S Signs and symptoms	Observations, symptoms, vitals and consciousness, with their source.	Synthetic heart rate; other assessments remain unknown until entered.
T Treatments	Treatments and interventions actually reported as performed.	No treatment assumed or automatically recommended.

Keep these observations separate

Observation	Required distinction
Heart rate	Show the bpm value with a simulated label. A high rate alone does not establish injury.
Blood pressure	Record the method and measured value: cuff systolic/diastolic or palpated systolic. Otherwise show Not measured.
Pulse assessment	Record Not assessed, Present or Absent, with a site when known. Pulse presence does not produce a BP estimate.
Consciousness	Use Not assessed, Alert, Responds to voice, Responds to pain or Unresponsive. Do not infer this from heart rate.
Treatments and source	Record only reported performed interventions. Retain the reporter and elapsed demo time.

Do not interpret a heart rate above 90 bpm as a systolic blood pressure above 90 mmHg. These are different measurements. A palpated systolic pressure must be obtained by measurement; an observed pulse alone is not an entered blood pressure.

Clinical and display boundaries

The anatomy panel uses the supplied human base mesh with rotation, zoom and view presets. It is the same generic model for each fictional officer. Until a validated observation provides an injury location, use a text report without placing a wound marker. These controls are for the hackathon demonstration, not clinical care.

The medical terminology was checked against MIST and AVPU references listed on the final page. Local EMS protocols and professional review would determine any real workflow.

Engineering and device integration

The dashboard uses Next.js, React, TypeScript, Mapbox GL JS and Three.js. The map has real basemap geometry, but its officer positions and routes are scripted. The supplied Blender Human Base Meshes 1.4.1 asset is CC0; the local GLB and license record remain with the app.

Preserve the team's existing work

The GitHub repository also contains a newer root app with live fleet tracking and a hazard-triage backend. The dashboard is a separate app under apps/paw-patrol, with its own npm lockfile and Node 22 toolchain. The root app keeps its existing pnpm and Node 26 requirements. Do not replace one application's dependencies or configuration with the other's.

A future adapter should connect agreed event types to the dashboard. Each event needs a person or device identity, source, timestamp, value or report, uncertainty and freshness. Model alerts, manual reports and measured readings must remain distinguishable. Until that adapter is implemented and verified, the dashboard's scenario is not the teammate backend's live state.

Hardware inventory and readiness

Equipment	Intended role	Dashboard readiness
iPhone 16 Pro Max	Camera, microphone and GPS; possible relay.	No capture or phone pairing in this frontend.
Apple Watch SE	Potential heart-rate source.	Generation and companion software must be confirmed; no live readings here.
Mac and Wi-Fi	Run development and future receiving services.	Local dashboard runs; Mapbox needs network access and WebGL.
Chargers and mount	Power and stable camera placement.	Bring phone/watch chargers and a suitable mount.
Optional power bank and Ethernet	Backup power or networking.	Verify adapters and venue support if used.

ATAK is a future integration

ATAK is an Android tactical platform. A real plugin or supported data connection requires a compatible SDK, deployment and security decisions, and device-level testing. This browser dashboard is not an ATAK plugin. The interface should identify it as planned or not connected rather than implying that positions or alerts are being shared.

Before connecting real people

Agree on consent, authentication, device identity, permissions, retention and access by role. Begin with one consented position event and its timestamp. Validate stale or missing data behavior before media or health information. Model evaluation and clinical review are separate workstreams, not features implied by a polished interface.

Run the dashboard and hand off the work

Use the dashboard branch and its nested application. The following commands are for the dashboard, not the separate root app. Use Node 22.23.2 and npm 11.6.2 for the dashboard toolchain.

```
git clone --branch codex/paw-patrol-dashboard \
  https://github.com/alex-nyan/hackmit_2026.git
cd hackmit_2026/apps/paw-patrol
npm ci
npm run setup
```

Enter your own public Mapbox token in .env.local as NEXT_PUBLIC_MAPBOX_ACCESS_TOKEN. Never commit that file or a secret token. If the token restricts URLs, allow the local host and chosen port. Then run:

```
npm run dev -- --port 5176
# Open http://localhost:5176
npm run verify
```

For a local production build, run npm run build and then npm start -- --port 5176. Set the public token before building, and stop the development server before reusing its port. Hosting on Vercel is a separate deployment step; point its project root to apps/paw-patrol and configure the environment before building.

Team handoff checklist

Frontend: run the three views and verify selection, reset, panic acknowledgement, scene gating and MIST validation. Integration: agree on the event contract with the existing tracking and backend owners. Devices: confirm the Watch SE generation, capture software and permissions. Demo owner: rehearse both the default sequence and the unsafe-scene hold, and explain which parts are simulated.

Before sharing, verify a clean installation, type checks, tests and a production build. In the browser, check map loading, body front/side/back views, controls, failure states and person switching. Publishing this branch must not change main or remove teammates' source files. GitHub Actions and the commit history provide the final build and publication status.

Sources and implementation references

[MIST communication example](#) - Joint Trauma System CoERCCC meeting materials.

[AVPU assessment](#) - Queensland Health consciousness terminology.

[Heart rate and blood pressure](#) - American Heart Association measurement distinction.

[ATAK CIV development](#) - TAK Product Center project documentation.

[Blender demo assets](#) - Human Base Meshes source; see public/models/LICENSE.md in the app.